ASHAMitra — AIIMS Kalyani Pitch Deck, Explained in Easy Words

> A plain-language, example-rich walkthrough of every slide in `ASHAMITRA_AIIMS_Kalyani.pptx` (17 slides). Every abbreviation is decoded the first time it appears; a full **cheat-sheet** is at the end.

What this deck actually is (read this first)

This is a **pitch deck** — slides used to *propose* something in a meeting.

- **Who is presenting:** a startup called **Flint De Orient** (founder: Sabir Ali Mollah, Kolkata).
- **Who they're presenting to: AIIMS Kalyani** — a top government medical college and hospital.
- **What they want:** for AIIMS doctors to (1) **check the app's medical rules are correct**, (2) **approve** them, and (3) **co-run a small real-world trial ("pilot")** in villages.
- **The product: ASHAMitra**, a phone app for **ASHA workers** (village health workers) that replaces their paper registers and helps them spot danger early.

One-line story of the whole deck: "*Village health workers drown in paperwork, so danger signs go unseen for weeks. We built one safe, offline, Bengali app to replace all that paper — please help us prove it's medically safe and test it with you.*"

> 🧠 **Who is an ASHA?** *Accredited Social Health Activist* — a trained local woman who is the health helper for her village (~1,000 people). She visits pregnant women, newborns, and children, gives advice, tracks vaccines, and refers sick people to hospitals. Today she does almost all of this on **paper**.

Slide 1 — Title page

What it says: Company name, product name, and presenter.

Every point, in easy words: - **FLINT DE ORIENT** — the company's name. - **DPIIT Recognised Startup** — *DPIIT* = the government's *Department for Promotion of Industry and Internal Trade*. It officially certifies real startups. So this badge means "*a government-recognised genuine startup.*" It's there to build trust. - *Example:* like a shop showing its government licence on the wall. - **CIN U72900WB2022PTC256145** — *Corporate Identity Number*: the company's unique government registration number (like an Aadhaar number, but for a company). "WB2022" tells you it was registered in West Bengal in 2022. - **ASHAMITRA — A digital workspace for ASHA workers** — the app's name and its core promise: *one digital "office" for everything the ASHA does* (not just one small feature). - **Co-designed for clinical safety** — "*built together with doctors so it is medically safe,*" not by software people alone. This is the message AIIMS most wants to hear. - **Product walkthrough and pilot discussion · AIIMS Kalyani** — "*today we'll (a) show you the app and (b) discuss running a trial.*" A *walkthrough* = a guided demo.

Slide 2 — "Where we started" (the problem)

What it says: Right now the ASHA carries the whole system on paper, and that causes dangerous delays.

Every point, in easy words: - **"The ASHA carries the system on paper."** — everything depends on her handwritten notebooks. - **~1,000 population per ASHA** — each ASHA looks after about 1,000 villagers, *across maternal, newborn, child, immunisation, NCD and reporting duties.* - **NCD** = *Non-Communicable Disease* — illnesses you don't "catch" from others: diabetes, high blood pressure, cancer. - **15+ paper registers** — she fills **more than 15 separate notebooks**. Examples given: - **Eligible couple register** — couples who could have a baby (for family-planning follow-up). - **MCP** = *Mother & Child Protection* card — the standard pregnancy + child health card. - plus **monthly reporting, immunisation, growth tracking** registers. - *Relatable example:* imagine your job forced you to copy the *same* customer's details into 15 different diaries by hand, every visit. That's her day. - **30 days to system visibility** — "*Lag between an event in the village and a record anyone can act on.*" It can take **a month** before information she records reaches anyone who can act on it — because data only moves up on the monthly "reporting day." - **The punchline:** "*Clinical alerts — newborn danger signs, ANC high-risk flags, defaulter immunisations — are buried in paper that no one sees until reporting day.*" - **ANC** = *Antenatal Care* (pregnancy check-ups before birth). **defaulter immunisation** = a child who missed a vaccine. - *Example:* a newborn showing danger signs today, or a mother flagged as high-risk, sits *invisible* in a notebook for weeks — and by reporting day it may be too late.

Slide 3 — "What the field visit told us" (the key decision)

What it says: After visiting the field, they realised a *half*-digital app would be **worse** than paper — so they chose to digitise **everything**.

Every point, in easy words: - **"The field visit changed our design."** — seeing real ASHAs at work changed their plan. - **"Partial digitisation creates worse outcomes than pure paper."** — if the app does only *part* of the job, she has to run **both** the app **and** paper at the same time. Two systems = double work, and she trusts neither. - *Relatable example:* if your phone calendar held only *some* of your meetings, you'd still keep a paper diary too — now you're juggling two unreliable lists instead of one. More mistakes, not fewer. - **"An app that handles only triage..."** — **triage** = sorting patients by urgency (who needs help first). An app that *only* does triage but not reporting would force the paper to keep running for audits. - **OUR RESPONSE — "Digitise the entire ASHA workflow end-to-end. Make paper genuinely redundant."** — replace **all** her paperwork so paper becomes truly unnecessary. *End-to-end* = from the first step to the last. - **"This reframes ASHAMITRA from a clinical decision support tool into a complete digital workspace — with clinical decision support as one layer inside it."** - *Translation:* "We're not just a medical-advice app anymore. We're the ASHA's whole digital office, and the medical-advice part is **one section inside it.**" - *Example:* like going from a single calculator app to a full office suite where the calculator is just one tool.

Slide 4 — "The product" (everything it does)

What it says: Lists the app's five main features and its safety design rules.

The five features, in easy words: 1. **Clinical decision support — 7 modules, deterministic, IMNCI/HBNC/HBYC-aligned** - This is the "give safe medical advice" part — 7 checklists (newborn, child, pregnancy, etc.). - **deterministic** = predictable: the *same answers always give the same result*, every time (no randomness, no guessing). Crucial for medical safety. - **IMNCI** = *Integrated Management of Neonatal and Childhood Illness* (the official method for sick young children). **HBNC** = *Home-Based Newborn Care*; **HBYC** = *Home-Based Young Child Care* — the scheduled home visits ASHAs make. "*Aligned*" = follows these official guidelines. 2. **Immunisation tracking — UIP schedule, due/missed/overdue, reminders** - **UIP** = *Universal Immunization Programme*, the national vaccine timetable. The app knows which vaccine is **due, missed, or overdue** and sends reminders. - *Example:* like a phone reminder for "car service due," but for a baby's next vaccine. 3. **Growth monitoring — Weight, height, MUAC with auto-flags** - **MUAC** = *Mid-Upper-Arm Circumference* — a coloured tape wrapped around a child's upper arm to detect malnutrition. "*Auto-flags*" = the app automatically raises a warning if a number is dangerous. 4. **Monthly HMIS reporting — Auto-generated from encounters; supervisor sync** - **HMIS** = *Health Management Information System* — the government's required monthly health report. The app **builds the report by itself** from the visits she already did, and syncs it to her supervisor. - *Example:* instead of spending two nights copying registers, the report writes itself and she just reviews it. 5. **Referral cascade tracking — The strategic data layer for research and policy** - **referral cascade** = the chain of events *after* she sends someone to a hospital (did they go? what happened?). Tracking this creates valuable data for research and government policy ("strategic data layer").

The design principles (their safety rules), in easy words: - **Bengali-first; voice and touch input** — the app works in Bengali first; she can **speak** to it or **tap** it (helpful if reading/typing is hard). - **Deterministic clinical logic — no LLM in the diagnostic path** — **LLM** = *Large Language Model* (the tech behind ChatGPT). They **deliberately do NOT** use ChatGPT-style AI to make medical decisions, because it can be unpredictable or "make things up." Diagnosis uses **fixed, written rules only.** - *Why it matters:* a doctor can read every rule and know exactly what the app will do — no surprises. - **Rules sourced from MoHFW / NHM / WHO guidelines** — **MoHFW** = India's *Ministry of Health & Family Welfare*; **NHM** = *National Health Mission*; **WHO** = World Health Organization. The medical rules are copied from these official sources, not invented. - **On-device PII; controlled sync for de-identified data** — **PII** = *Personally Identifiable Information* (name, Aadhaar, phone). Personal details stay **on the phone**; only **de-identified** data (names removed) is sent out, and only carefully. - *Example:* the village keeps the names; the outside world only sees "a 25-year-old pregnant woman in Block X," never "Rita Das." - **Runs on the ASHA's own entry-level Android** — works on the **cheap basic phone she already owns** (no expensive device to buy). - **Auditable decision trace per case** — for every patient, the app saves a record of **why** it decided what it did, so it can be checked later.

Slide 5 — "MVP · Clinical layer" (the 7 modules)

What it says: The first version has seven medical checklists.

- **MVP** = *Minimum Viable Product* — the **first, smallest working version** you release to learn from real use. (Not the final product — the starting point.)

The 7 modules, in easy words: - **Newborn (0–28 days)** — *HBNC + IMNCI · PSBI · jaundice · feeding.* **PSBI** = *Possible Serious Bacterial Infection* (a dangerous newborn infection that can kill within hours); **jaundice** = yellow skin/eyes; feeding problems. - **Infant (29 days – 11 mo)** — *IMNCI Sick Young Infant* rules (the official checklist for sick babies in this age range). - **Child (1–5 years)** — *IMNCI + HBYC.* -

Pregnancy (ANC) — *PMSMA HRP + WHO ANC 2016.* **PMSMA** = *Pradhan Mantri Surakshit Matritva Abhiyan* (a government safe-pregnancy scheme); **HRP** = *High-Risk Pregnancy*; **WHO ANC 2016** = WHO's 2016 pregnancy check-up guidelines. - **"Dual-track: hard-stop + cumulative risk score"** — **two ways** to catch danger at once: a **hard-stop** (one serious sign = instant alarm) **and** a **cumulative risk score** (several small worries adding up to a warning). - **Postpartum (PNC, 0–42 days)** — *PPH (pad-soak proxy) · eclampsia · sepsis.* **PNC** = *Postnatal Care*; **PPH** = *Postpartum Haemorrhage* (heavy bleeding after delivery — the #1 killer of new mothers); **"pad-soak proxy"** = estimating blood loss by counting how many sanitary pads get soaked; **eclampsia** = dangerous pregnancy fits; **sepsis** = infection spreading through the body. - **Immunisation (Birth – 16 years)** — *UIP schedule · catch-up rules · reminders.* (catch-up rules = how to safely give missed vaccines late.) - **Emergency (Cross-cutting)** — *Unified severity 1–5 registry · referral composer.* **"Cross-cutting"** = applies to any patient. **"severity 1–5"** = rates the emergency from 1 (mild) to 5 (critical). **"referral composer"** = helps the worker put together the referral note.

The key safety behaviour: - **"Every module runs a universal danger-sign pre-sweep before tile-specific questions."** — a **tile** is a module button on the screen. Before the detailed questions, the app does a quick **scan for any life-threatening sign** first. - **"RED conditions terminate the flow and emit a referral immediately."** — if something is **RED** (emergency), the app **stops asking more questions** and **immediately** gives a "refer now" instruction. - *Relatable example:* a triage nurse who, the moment she sees you can't breathe, drops the paperwork and rushes you straight in.

Slide 6 — "MVP · The data layer" (tracking referrals)

What it says: For every patient the ASHA refers, the app records the whole journey — to fix a problem nobody currently sees.

What it captures per referral, in easy words: - **Decision timestamp** — the exact time she decided to refer. - **GPS at the decision point** — her location at that moment (to map where referrals happen). - **Recommended facility level (PHC / CHC / FRU / SNCU)** — which level of hospital she's sending them to: - **PHC** = *Primary Health Centre* (nearest small clinic), **CHC** = *Community Health Centre* (bigger), **FRU** = *First Referral Unit* (can do C-sections, blood transfusions, emergencies), **SNCU** = *Special Newborn Care Unit* (a newborn ICU). - **Patient/family acceptance — or refusal** — did the family agree to go, or say no? - **Refusal reason (transport, distance, family, cost, mistrust)** — *why* they refused. - **Outcome at facility (admitted, turned away, sent elsewhere)** — what actually happened when they reached the hospital. - **Time-to-care interval (decision → arrival → care initiated)** — how long from "decided to refer" to "actually got treated."

Why this matters, in easy words: - **"The community-to-facility link is currently a black box."** — **"black box"** = nobody can see inside it. Today, once the ASHA refers someone, **no one knows** if they actually reached the hospital. - *Example:* she tells a mother with danger signs "go to the hospital now," but the mother may never go (no transport, no money, family says no) — *and the ASHA never finds out something went wrong.* - **"Closes the loop on every referral"** — makes sure the **outcome comes back** for each referral ("closing the loop"). - **"Surfaces refusal patterns while they're still actionable"** — shows *why* people refuse, **early enough to fix it** (e.g., "most refusals are about transport in this village → arrange transport"). - **"Gives supervisors real-time visibility into referral gaps"** — bosses see problems **live**, not a month later. - **"Supports MDSR / audit review when needed"** — **MDSR** = *Maternal Death Surveillance and Response*: the official review of **every** mother's death to learn what failed. This data feeds those reviews.

Slide 7 — "Clinical safety" (why a top hospital can trust it)

What it says: Six promises that make it **"AIIMS-grade"** (good enough for AIIMS standards).

Every point, in easy words: - **Deterministic logic only** — "*No generative AI in any clinical decision pathway. Every classification is rule-based and reproducible.*" No ChatGPT-style AI deciding medical things; every result comes from fixed rules and can be repeated exactly. - **Guideline-sourced rules** — "*IMNCI, HBNC, HBYC, PMSMA, UIP, WHO ANC 2016. Every rule cites its source.*" Each rule says **which official guideline it came from** — like footnotes in a research paper. - **Committee sign-off gate** — "*No clinical rule reaches production JSON without AIIMS committee approval. App refuses to load unsigned bundles.*" - **JSON** = the file format the rules are stored in (in your app this is literally `asha_engine.json`). A **doctors' committee must approve every rule** before it goes live, and the app **refuses to run** an unapproved (unsigned) rule file. - *Example:* like a pharmacy that won't dispense a medicine until a doctor has signed the prescription. - **Auditable trace** — "*Each case carries protocol hash; rule version is reproducible months later for MDSR audit.*" A **protocol hash** is a unique fingerprint of *which version of the rulebook* was used, so months later you can prove exactly which rules decided a case. - **Encrypted data path** — "*AES-256 at rest, TLS 1.3 in transit.*" **AES-256** = strong lock for data **stored** on the phone ("at rest"); **TLS 1.3** = strong lock for data **traveling** over the internet ("in transit"). - *Example:* AES-256 is the locked safe; TLS 1.3 is the armoured van that carries it between buildings. - **IEC-led** — "*Pilot starts only after AIIMS Kalyani IEC clearance. DPDP Act 2023 compliant.*" **IEC** = *Institutional Ethics Committee* — the board that must approve any study involving people. **DPDP Act 2023** = India's *Digital Personal Data Protection Act* (the privacy law). So: **no trial until ethics approval**, and it obeys the privacy law.

Slide 8 — "Phase 2 — Post-pilot" (what gets added later)

What it says: *After* the first trial succeeds, they'll add more medical areas — but always with the same careful approval process.

Every point, in easy words: - **"After MVP proves out, we widen clinical breadth."** — once the first version works, add more topics. - **"Each Phase 2 module follows the same governance pathway: clinical committee sign-off, IEC amendment, then rollout."** — every new area must **again** get doctor approval + ethics approval before release. **"Discipline is the point — scope expansion never compromises the safety architecture."** = "*Getting bigger will never make it less safe.*" - **The future modules:** - **TB screening — NTEP** — **TB** = tuberculosis; **NTEP** = *National TB Elimination Programme*. - **NCD screening — CBAC-based** — **CBAC** = *Community-Based Assessment Checklist* (a standard form to spot diabetes / BP / cancer risk). - **Mental health — PHQ-2 / GAD-2** — two tiny questionnaires: **PHQ-2** screens for depression, **GAD-2** for anxiety. - **MUAC malnutrition — SAM / MAM in under-5s** — **SAM** = *Severe Acute Malnutrition*, **MAM** = *Moderate Acute Malnutrition* (very thin / thin children under 5). - **Adolescent health — RKSK / WIFS** — **RKSK** = *Rashtriya Kishor Swasthya Karyakram* (national teenager-health programme); **WIFS** = *Weekly Iron and Folic Acid Supplementation* (weekly iron tablets for teens).

Slide 9 — "Phase 2 in depth" (details + why each fits)

What it says: Explains each future module and notes that the ASHA *already* has these duties — the app just digitises them.

Every point, in easy words: - **TB screening (NTEP)** — "*Cough > 2 weeks + cardinal-sign assessment + household contact tracing → sputum referral.*" - If someone coughs **more than 2 weeks**, check the **main TB signs** ("cardinal signs"), check **who else in the house** might be infected ("contact tracing"), then send them for a **sputum** (spit) test. "*ASHAs already have explicit NTEP duties; we structure and digitise them.*" — she already does this on paper. - **NCD screening (CBAC)** — checklist for **adults 30+**, screening **diabetes, high BP, and oral / breast / cervical cancer** risk. "*Aligns with Ayushman Bharat HWC roles already in place.*" — **HWC** = *Health & Wellness Centres* under the Ayushman Bharat scheme; she already has these duties there. - **Mental health screen** — **PHQ-2 (depression)** and **GAD-2 (anxiety)** — **Bengali-validated** (proven to work in Bengali), **two-question** screens. "*Brief, low-burden*" = very quick, almost no extra work. "*Emerging ASHA role under DMHP*" — **DMHP** = *District Mental Health Programme*. "*Underserved evidence space*" = little research exists here, so a chance to contribute something new. - **MUAC malnutrition** — arm-tape measure for **children 6–59 months** (6 months to 5 years), with SAM/MAM thresholds and auto-referral. "*Already paper-based — digitisation is straightforward*" = easy to convert. - **Adolescent health (RKSK / WIFS)** — weekly iron-tablet tracking, **menstrual hygiene counselling**, and teenage **anaemia** (low blood) screening. "*Formal ASHA duties, currently poorly tracked.*"

Slide 10 — "Phase 3 · Research frontier" (the ambitious future)

What it says: Long-term, AI-assisted ideas they'd build **with AIIMS leading the science.** "*Research frontier*" = cutting-edge, not yet proven.

Every point, in easy words: - **AI-assisted visual triage — Phone-camera screening** — using the phone **camera** to screen for: - **Oral lesions (cancer screen)** — mouth patches/sores that could be mouth cancer. - **suspicious skin (leprosy, moles)** — skin patches (leprosy) or moles (skin cancer). - **diabetic foot ulcers** — foot wounds in diabetics that can turn dangerous. - **wound infection grading** — judging how badly a wound is infected. - "*Uses the ASHA's own phone camera, no additional hardware.*" — no special equipment needed. - "*Image triages locally; flagged cases hand off to CHO/PHC.*" — the phone checks the photo **itself** and only passes worrying cases upward. **CHO** = *Community Health Officer* (runs the wellness centre). - "*Brings specialist-level screening within ASHA reach — early detection where it currently doesn't happen.*" — catches things early in villages that have no specialists today. - **Diabetic retinopathy — Specialist hand-off model** — **Diabetic Retinopathy (DR)** = eye damage caused by diabetes, which can cause blindness. - "*Honest framing*" — they are being **upfront about a limitation** (a sign of credibility to doctors). - "*fundus imaging needs either a hardware attachment (Remidio-class) or dilated retinal photography — neither fits an ASHA door-to-door workflow.*" — **fundus** = the back inside of the eye; photographing it needs a **special camera** (**Remidio** is a brand of portable eye camera) or **eye-dilating drops** — neither is practical for an ASHA walking house to house. - "*Proposed model: ASHAMITRA flags diabetic adults via the NCD module; CHO-level fundus imaging + AI grading + ophthalmologist confirmation. AIIMS would lead the clinical workflow design.*" — the app flags diabetics → eye photos taken at the CHO level → **AI grading** → an **ophthalmologist** (eye doctor) confirms. AIIMS designs the medical process.

Slide 11 — "Workflow impact" (what changes for the ASHA)

What it says: A before/after table showing how much **time and effort the app saves her.**

Task	Today (on paper)	With ASHAMitra
Field-visit documentation	~30 min/visit on registers, then **re-copied at night**	**voice + structured capture in ~5 min**, saved as she works
Monthly HMIS report	**2 evenings** compiling register-by-register	**auto-made** from visits she already did; ~10-min review
High-risk pregnancy detection	from memory + register flags; **often missed until late**	a **risk score at every ANC visit** + danger-sign pre-sweep
Closing the loop on referrals	she refers and waits; **outcome rarely comes back**	**cascade tracked**; alerts if the patient doesn't arrive; outcome returned to her

- **The key promise:** *"We're not asking the ASHA to do more work. We collapse what she already does into a fraction of the time — and surface what currently slips through."*
- *Translation:* this is **not extra work** — it makes her **existing** work faster, and it **catches things she currently misses.**
- *Relatable example:* like a shopkeeper switching from handwritten account books to a billing app — same sales recorded, but in minutes, and it automatically flags who hasn't paid.

Slide 12 — "Health impact" (what changes for the village)

What it says: The real health benefits, area by area.

Every point, in easy words: - **Newborn** — PSBI, jaundice, and feeding problems are **caught at the visit** — not weeks later at a supervisor review or, worst case, at a **mortality audit** (a death review). - **Pregnancy** — high-risk pregnancies are flagged **from the very first ANC visit**; eclampsia and PPH danger signs trigger a **structured immediate referral.** - **Immunisation** — *"Defaulter list runs every morning."* A **defaulter** is a child who missed a vaccine; each morning the app lists them, mothers get reminders, and supervisors see the gap **in real time.** - **Referral cascade** — refusal reasons (transport, distance, family, cost) **become visible**, so the system can fix the **real cause**, not just the symptom. - **Growth & nutrition** — MUAC and weight tracked at **every under-5 visit**; SAM/MAM caught **in the village**, not at the **quarterly AWC review**. **AWC** = *Anganwadi Centre* (the village child-care + nutrition centre). - **The summary line:** *"The system already says these things should happen. ASHAMITRA makes them happen consistently, on time, with proof — at the village level."* - *Translation:* the rules already exist on paper; the app makes them **actually happen, reliably, with evidence.**

Slide 13 — "Pilot proposal" (the trial plan)

What it says: The concrete plan for the trial run.

Pilot shape, in easy words: - **Site** — 1 district near AIIMS Kalyani, chosen **together**. - **Cohort** — **"cohort"** = the group of participants; about **20 ASHAs**, final number with AIIMS's input. - **Duration** — **4–6 months** of active use. - **Onboarding** — **"onboarding"** = getting them set up and trained: **4–6 weeks** of training plus a supervised **baseline** (measuring the starting situation *before* the app, so improvements can be measured later). - **Clinical oversight** — **monthly AIIMS review** + an **emergency consultation channel** (a fast way to reach doctors if something serious comes up).

What they'll observe (measure): - ASHA **workflow time savings** (visits, reporting). - **Clinical decision quality** on AIIMS-reviewed cases. - **System uptime** (how reliably the app runs without failing), **sync reliability**, **offline behaviour**. - Feedback from supervisors and the **ANM** — **ANM** = *Auxiliary Nurse Midwife*, the nurse who supervises ASHAs. - **Field-level acceptance** (do the ASHAs actually like and use it?). - **Issues, edge cases, content gaps** — **edge cases** = rare/unusual situations; **content gaps** = missing rules — all noted for improvement.

Slide 14 — "What we're asking" (the specific request)

What it says: Exactly what they want AIIMS to do.

Every point, in easy words: - **Clinical review** — check the 7 MVP modules against IMNCI, HBNC, HBYC, PMSMA, WHO ANC and **tell them what to change.** - **Sign-off gate** — **approve** the rules before they go live; no rule reaches the field without committee approval. - **Pilot oversight** — **monthly clinical review** during the pilot + **re-checking a sub-sample** (a small selection) of cases + an **open emergency consult line** throughout. - **Site & cohort** — help pick the **right district and ASHA group** near AIIMS Kalyani. - **Phase guidance** — tell them **which Phase-2 module to do first**, and whether the **Phase-3 DR** (diabetic retinopathy) idea makes sense ("resonates"). - **Field & training input** — share AIIMS teaching material they can adapt for **ASHA training**, and point out what the field team would want refined.

Slide 15 — "Indicative timeline" (rough 12-month plan)

What it says: A ~12-month plan, from review to a "should we scale up?" decision. **"Indicative"** = approximate, not fixed.

Phase	Months	What happens
Phase 1 — Clinical review & sign-off	1–2	AIIMS reviews content, committee **approves**, training material adapted, pilot site chosen
Phase 2 — Training & baseline	3–4	ASHAs trained, **baseline** data captured, devices handed out, first supervised use
Phase 3 — Active deployment	5–9	full pilot running, **monthly AIIMS review**, ongoing support, **iterative refinement** (improving in repeated cycles)
Phase 4 — Review & decide	10–12	joint review, final tweaks, **"go / no-go decision on scale-up"** = decide whether to roll out widely or stop

- **"Timeline assumes clinical review concludes within 6–8 weeks. Phase boundaries are indicative."** — this schedule depends on the review finishing in ~6–8 weeks; the dates are approximate.

Slide 16 — "Open questions" (decisions for AIIMS)

What it says: Six specific questions to start the discussion.

1. **Clinical priority** — which module should AIIMS validate **first** — Newborn, ANC, or Immunisation?
2. **Site selection** — which nearby district is most **feasible** (practical) for the first pilot?
3. **Review cadence** — **"cadence"** = how often; monthly reviews, or **weekly** during the early weeks?
4. **Cohort size** — 15, 20, or 25 ASHAs? *"Smaller and tighter, or wider for variance?"* — fewer people (easier to manage closely) vs more people (more **variety/variance** in the data).
5. **Phase 2 priority** — which next module best matches AIIMS priorities — TB, NCD, mental health, MUAC, or RKSK?
6. **Phase 3 direction** — does the **DR specialist hand-off** model fit AIIMS's workflow? Or other camera-screening ideas first?

Slide 17 — "Thank you"

What it says: Closing message + contact.

- **"We're listening."** — signalling they're open to feedback and partnership.
- **"The product we build with you is the product that's safe enough to use across rural West Bengal."** — *"by building it together with AIIMS, it becomes safe enough to use widely."* (A respectful, collaborative close.)
- **Sabir Ali Mollah · Founder, Flint De Orient** — the presenter.
- **DPIIT Certificate No. DIPP265928** — the startup-recognition certificate number (the proof behind the "DPIIT Recognised" badge on slide 1).

📖 One-page cheat-sheet — every abbreviation in the deck

Short form	Full form	In easy words
ASHA	Accredited Social Health Activist	The trained village health worker (a local woman) this app is for.
CDSS	Clinical Decision Support System	Software that helps decide the right medical action.
MVP	Minimum Viable Product	The first small working version, released to learn from.
DPIIT	Dept. for Promotion of Industry & Internal Trade	Govt body that certifies real startups.
CIN	Corporate Identity Number	A company's unique govt registration number.
NCD	Non-Communicable Disease	Diabetes, high BP, cancer — illnesses you don't catch.

Short form	Full form	In easy words
MCP	Mother & Child Protection (card)	The standard pregnancy + child health card.
HMIS	Health Management Information System	The govt's monthly health report.
ANC	Antenatal Care	Pregnancy check-ups *before* birth.
PNC	Postnatal Care	Mother/baby check-ups *after* birth.
IMNCI	Integrated Management of Neonatal & Childhood Illness	Official method for sick young children.
HBNC / HBYC	Home-Based Newborn / Young-Child Care	The ASHA's scheduled home visits.
PMSMA	Pradhan Mantri Surakshit Matritva Abhiyan	Govt safe-pregnancy (free ANC) scheme.
HRP	High-Risk Pregnancy	A pregnancy that needs closer watching.
UIP	Universal Immunization Programme	The national vaccine schedule.
PSBI	Possible Serious Bacterial Infection	Dangerous newborn infection that worsens fast.
PPH	Postpartum Haemorrhage	Heavy bleeding after delivery (top killer of mothers).
Eclampsia	—	Dangerous fits in/after pregnancy from high BP.
Sepsis	—	Infection spread through the whole body.
MUAC	Mid-Upper-Arm Circumference	Arm-tape test for child malnutrition.
SAM / MAM	Severe / Moderate Acute Malnutrition	Very thin / thin child under 5.
MDSR	Maternal Death Surveillance & Response	Official review of every mother's death.
IEC	Institutional Ethics Committee	Board that approves research on people.
DPDP Act 2023	Digital Personal Data Protection Act	India's data-privacy law.
PII	Personally Identifiable Information	Name, Aadhaar, phone — personal details.
LLM	Large Language Model	ChatGPT-style AI (deliberately NOT used for diagnosis).
AES-256 / TLS 1.3	encryption standards	Strong locks for stored / traveling data.
NTEP	National TB Elimination Programme	India's TB programme.
CBAC	Community-Based Assessment Checklist	Form to spot diabetes/BP/cancer risk in adults 30+.
PHQ-2 / GAD-2	2-item depression / anxiety screens	Two quick questions each.
DMHP	District Mental Health Programme	Govt mental-health programme.
RKSK / WIFS	Adolescent health programme / Weekly Iron-Folic Acid	Teen health + weekly iron tablets.
DR	Diabetic Retinopathy	Diabetes-caused eye damage (can blind).
CHO	Community Health Officer	Runs the Health & Wellness Centre, above the ASHA.
ANM	Auxiliary Nurse Midwife	The nurse who supervises ASHAs.
AWC	Anganwadi Centre	Village child-care + nutrition centre.
HWC	Health & Wellness Centre	Upgraded clinic under Ayushman Bharat.
PHC / CHC / FRU / SNCU / DH	Primary / Community Health Centre · First Referral Unit · Special Newborn Care Unit · District Hospital	Facility levels, smallest → biggest (SNCU = newborn ICU).